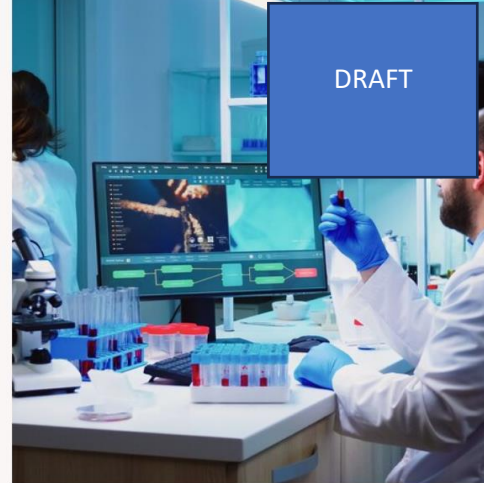




SECTOR AT A GLANCE | 01

Health Sector in India

An Overview

DRAFT

Sector at a Glance 01

Health Sector in India

An Overview

The '**Sector-at-a-Glance**' provides an easy-to-read overview and basic understanding of a sector, sub-sector, or thematic area. The facts and insights on the sector are supported by relevant data and best practices from credible government and multilateral sources. The document covers the structure, key trends, opportunities and challenges, government initiatives, and the way forward for the sector.

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Overview

Over the past decades, India has achieved notable advancements in the realm of healthcare, including an increase in life expectancy, reductions in fertility rates, and significant declines in maternal and child mortality rates. These achievements are a testament to dedicated initiatives and programmes undertaken by the government over the years. The National Health Policy 2017 is the most important health policy action taken by the present Union Government. The NHP aims to achieve the universal, equitable, affordable, accountable, patient-oriented quality of care, professionalism and integrity, inclusive partnerships and pluralism, decentralisation and dynamism, and adaptability, which are the pillars of universal health coverage (UHC). The NHP 2017 provides a clear framework for UHC. It calls for good health status with prevention and promotion, as well as a focus on the quality and affordability of primary care.²

Moreover, there has been a discernible increase in the contribution of the public sector to healthcare in the last five years, aimed at fulfilling the targets outlined in the Sustainable Development Goals (SDGs).¹ Although public health is constitutionally a state subject in India, the Central Government has actively shaped the country's healthcare system through various national programmes and initiatives.¹ The Ministry of Health and Family Welfare plays a significant role in the sector by establishing national health policies and overseeing centrally sponsored schemes, central sector schemes, and research and development institutions. State governments provide two-thirds of the funding for public health expenditure, with the remaining one-third coming from the Central Government.²

Evolution of the Healthcare Sector in India

Before Independence: Traditionally, healthcare in India has relied on voluntary efforts. Throughout history, traditional healthcare practitioners have played a crucial role in meeting society's medical needs. Profound knowledge about the medicinal qualities of plants and herbs has been passed down through generations for therapeutic purposes. However, the landscape shifted during the colonial era, particularly under British rule. Christian missionary-managed hospitals gained prominence, and even India's intellectual elite, influenced by a pro-Western bias, began favouring Western medical practices.³

After Independence: Before gaining independence, healthcare in India was in a state of disarray, marked by a high mortality rate and the rampant spread of infectious diseases. Following independence, the Indian government prioritized Primary Health Care and has consistently worked towards improving the healthcare system nationwide. Nevertheless, the government's efforts alone proved insufficient to meet the needs of a burgeoning population, whether in primary, secondary, or tertiary healthcare. Alternative sources of funding became crucial for the sustenance of the health sector.³

Entry of Private Sector: Around two decades ago, the private healthcare sector was primarily comprised of individual practitioners, small hospitals, and nursing homes. Notably, the quality of services, particularly in hospitals operated by charitable trusts and religious foundations, was outstanding. It was in the 1980s that the government, recognising its limitations in providing healthcare services on its own, began permitting the private sector to bridge the gap between healthcare supply and demand. Private hospitals are now overseen by corporate entities, non-profit

¹ Development Monitoring and Evaluation Office (DMEO) (2021). *Health Sector Report*.

² Asia Pacific Observatory on Health Systems and Policies, WHO (2022). *India Health System Review*.

³ Sheeba A. (2020). *Health Care System in India : An Overview*.

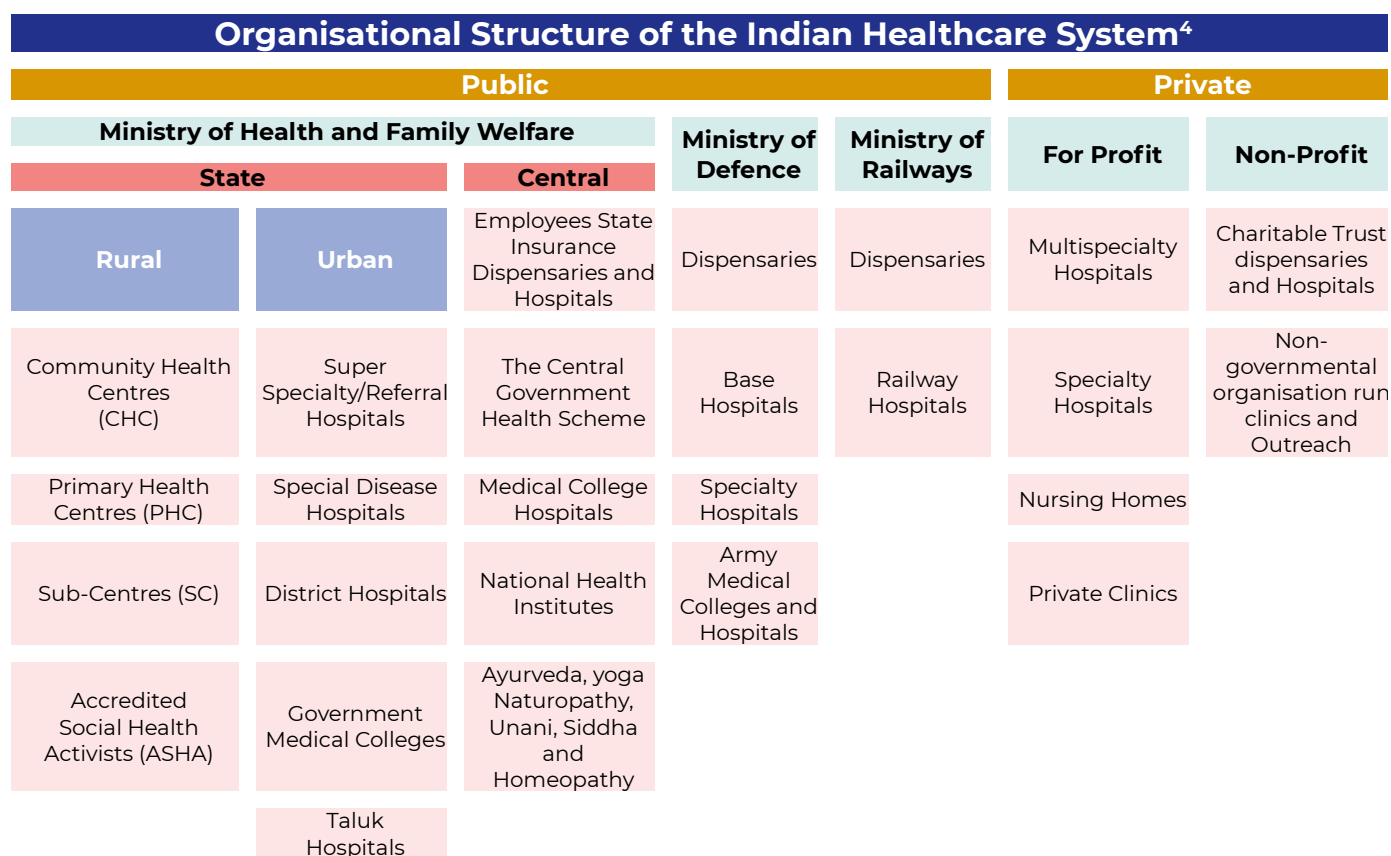
organisations, or charitable groups. The introduction of the private sector has opened various opportunities in areas such as medical equipment, information technology in healthcare services, business process outsourcing (BPO), and medical tourism.³

Healthcare System in India

Traditional Healthcare System: In India, alongside the presence of modern medicine, traditional and indigenous healthcare practitioners coexist and practice across the nation. These indigenous healthcare traditions encompass Ayurveda, Siddha, Unani, Naturopathy, and Yoga. Beyond these established traditional systems of medicine, there are also faith-based and spiritual healers, predominantly found in the rural regions of India. Some spiritual leaders incorporate one or more of the traditional therapies, such as Ayurveda, Yoga, or Naturopathy, as part of their healing methods when tending to their clients.³

Modern Healthcare System: India's modern allopathic healthcare system is comprising of three main components - the public sector, the private sector, and an informal network of care providers. Given the immense size, scale, and reach of the country, there has been commendable progress in adhering to well-intentioned guidelines and regulations. While established norms exist, continuous efforts are being made to enhance compliance. While there exists variation in the standards of services that can be offered, who can provide them, and the methods of delivery throughout the nation, continuous improvement is underway.³

Fig 1 - Organisational Structure of the Indian Healthcare System



Public healthcare system in India comprises of facilities operated by both the central and state governments. These public institutions offer healthcare services at no cost or reduced rates, primarily benefiting lower-income families in both rural and urban areas. The Indian Constitution allocates health-related responsibilities between the central and state governments. The central government is responsible for medical research and technical education, whereas state governments manage infrastructure, employment, and service delivery. Furthermore, the concurrent list, located in the 9th schedule of the Indian Constitution, deals with matters that affect multiple states, such as preventing the spread of infectious or contagious diseases across state boundaries. While states enjoy a significant degree of

⁴ MoHFW. (2023). *Exemplars in Maternal and Newborn Health India Study*.

autonomy in managing their healthcare systems, the central government wields substantial fiscal control over the states' healthcare systems.² To enhance the quality and delivery of health services, the National Health Policy 2017 suggests establishing a multidisciplinary public health management cadre (PHMC) in all states/UTs. The PHMC would include participants from different sectors and fields and would have well-defined career structures and diagrams. Similarly, 'Kayakalp Award Scheme' started on May 15, 2015 seeks to enhance and encourage the practices of cleanliness, hygiene, waste management and infection control in public health care facilities and reward the outstanding ones.



The Central Government takes charge of policy formulation, planning, direction, support, assessment, and synchronisation of the activities carried out by state health bodies²



As health is under the jurisdiction of individual states, each state manages its own healthcare infrastructure²

- It provides financial support for national health initiatives, aiming to guarantee sufficient access to healthcare services and uniformity in the execution of services among various states.
- National initiatives like the NRHM strive to establish fairness across states in terms of public health funding and the provision of services. They achieve this by offering greater funding to states that face challenges in performance and resource generation due to economic disparities.
- Within each state, the governmental healthcare system is structured within the framework of the Department of Health and Family Welfare, overseen by a state minister responsible for Health and Family Welfare, as well as a state minister responsible for Medical Education.
- States serve as the primary entities responsible for providing healthcare-related services, taking charge of on-the-ground programme implementation, and carrying the major portion of healthcare expenditure.

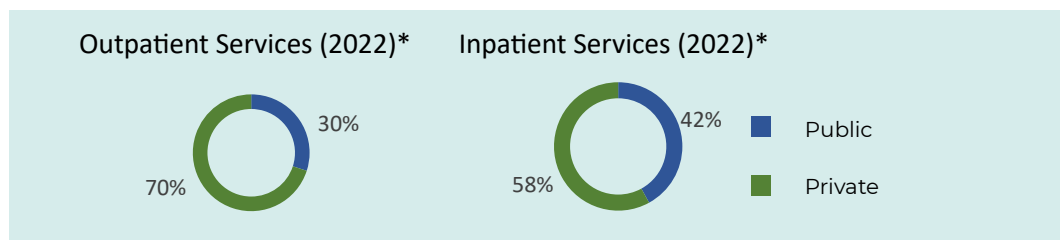
Private healthcare system in India encompasses a wide spectrum, including high-end corporate hospitals primarily situated in major urban areas, outpatient clinics in both large and small towns, as well as semi-qualified or unqualified medical practitioners. Additionally, the private sector offers diagnostic services and pharmaceuticals. While there are a few private non-profit or mission hospitals that offer quality secondary or tertiary care to patients either for free or at a reduced cost, such institutions are relatively scarce. The private healthcare sector in India has witnessed impressive growth over the years. Given the dominant presence of the private sector in the healthcare industry, several state governments in India have been exploring the option of engaging the private sector and forming partnerships with it to address the increasing healthcare demands of the population. In India, the private healthcare sector, which holds a significant share in healthcare delivery, encompasses general practitioners, as well as registered medical professionals, along with chemists and diagnostic laboratories.²

Payment for accessing healthcare services from private providers primarily involves out-of-pocket expenses and insurance funding. While privately purchased or publicly funded insurance schemes can offer partial or full coverage for hospitalization at approved facilities, most private outpatient care is typically paid for directly by individuals. Currently, private providers are subject to regulation through various measures, including the Clinical Establishments Act and Rules. The accreditation of facilities and clinical practices was previously overseen by the Medical Council of India (MCI) until it was succeeded by the National Medical Commission (NMC) in 2020. While government hospitals are established in accordance with Indian public health standards, there is a growing emphasis on seeking accreditation from organizations like the National Accreditation Board for Hospitals & Healthcare Providers (NABH).²

Government-funded healthcare services play a significant role in addressing healthcare needs, encompassing aspects such as preventive healthcare, promotion, medical education, and a substantial portion of outpatient and inpatient services. There have been notable enhancements in the public sector's

contribution since 2004, attributed in part to the initiatives and investments made through the National Health Mission (NHM).²

Fig 2 - Percentage use of public and private facilities for inpatient and outpatient care²



*Data is taken from latest WHO's report on Indian Health System review which was published in 2022

Health Infrastructure³

Health infrastructure serves as a vital metric for comprehending a nation's healthcare delivery systems and the strategies employed. It also reflects the investments made and the level of importance attributed to establishing healthcare facilities in both the public and private sectors. In India, the health infrastructure is extensive, covering various medical systems like allopathic, ayurveda, siddha, Sowa Rigpa, unani, and homeopathy. It can be classified as follows:

- Physical infrastructure
- Human resources

Physical infrastructure includes healthcare facilities offered by both the public and private sectors. Public health services comprise a network of sub-centres, primary health centres (PHCs), community health centres (CHCs), and district hospitals.

The infrastructure in the private sector provides for at least 80% of health services in the country and can be classified as follows:

- Private dispensaries
- Private hospitals
- Charity hospitals, including medical centres managed by NGOs
- Corporate hospitals

Despite an extensive public health care infrastructure, the private sector dominates the market

Human Resources: Healthcare professionals, which include surgeons, general practitioners, nurses, paramedical, allied health professionals, doctors, dentists, AYUSH practitioners, and others, engage in a variety of work arrangements. Within the public sector, they may hold regular government positions or work as contractual employees. Indian government telehealth is indeed a promising initiative to address the issue of doctor absenteeism in government healthcare facilities. It involves not only increasing doctor availability but also ensuring their consistent presence and commitment to serving rural communities. The practice of public doctors working in both the public and private sectors is considered a significant contributor to the growth of the private healthcare sector, particularly in rural areas. In the private sector, doctors may practice independently, within private hospitals, or, though less commonly, in group practices.

Major Developments in Progress of Health Programmes

In India, a variety of providers at the national and state levels are responsible for delivering essential public health services, which encompass programmes like population and epidemic surveillance, vaccination, family planning services, and preventive services. While the majority of public health services are government-funded and government-operated, there is also some involvement from private healthcare providers and non-governmental organisations (NGOs). At the national level, these public health services are structured within national health programmes, with many of them falling under the National Health Mission (NHM). State health departments play a pivotal role in executing these national health programmes, overseeing specific initiatives and services aimed at addressing priority areas.⁵

National Health Mission



The Ministry of Health and Family Welfare is implementing various schemes programmes and National initiatives to provide universal access to quality health care. The approach is to increase access, make affordable to the decentralised health system by establishing new infrastructure in deficient areas and by upgrading the infrastructure in existing institutions. As part of the planning process, many different programmes have been brought together under the overarching umbrella of National Health Mission with NRHM and NUHM as its two sub-Missions.

RMNCH+A Strategy



The RMNCH+A Strategy is a framework based on provision of comprehensive care through the five pillars, or thematic areas, of reproductive, maternal, neonatal, child and adolescent health, and is guided by central tenets of equity, universal care, entitlements, and accountability. The aim is to reach the maximum number of people in the remotest corners of the country through a continuum of services, constant innovation and routine monitoring of interventions.

Family Planning Programme



Family Planning Programme: India was the first country in the world to have launched a National Programme for Family Planning in 1952. Over the decades, the programme has undergone transformation in terms of policy and actual programme implementation and currently being repositioned to not only achieve population stabilization goals but also promote reproductive health and reduce maternal, infant and child mortality and morbidity.

Ayushman Bharat



Ayushman Bharat is an attempt to move from a selective approach to health care to deliver a comprehensive range of services spanning preventive, promotive, curative, rehabilitative, and palliative care. It has two components which are complementary to each other. Under its first component, 1,50,000 Health and Wellness Centres (HWCs) will be created to deliver Comprehensive Primary Health Care, that is universal and free to users, with a focus on wellness and the delivery of an expanded range of services closer to the community. The second component is the Pradhan Mantri Jan Arogya Yojana (PM-JAY) which provides health insurance coverage of Rs. 5 lakh per year to over ten crore poor and vulnerable families seeking secondary and tertiary care.

Fig 3 - Timeline of Introduction of Disease Programmes in India

1950s-1960s	• Family Planning Programme (1952) • National Leprosy Eradication Programme (1955) • National Filaria Control Programme (1955) • National TB Control Programme (1962) • National Iodine Deficiency Disorder Control Programme (1962)	
1970s-1980s	• National Programme for Prevention and Control of Cancer (1975) • National Programme for Control of Blindness (1976) • Universal Immunization Programme (1978) • National Cancer Registry Programme (1982) • National Mental Health Programme (1982) • National Guinea Worm Eradication Programme (1983) • Drug De addiction Programme (1988)	
1990s	• National Vector Borne Disease Control Programme (1990) • National AIDS Control Programme (1992) • Pulse Polio Immunisation programme (1995) • Yaws Control Programme (1996) • National Programme for Control and Treatment of Occupational Diseases (1998)	
2000s	• Integrated Disease Surveillance Projects (2004) • National Health Mission (2005) • National Programme for Prevention and Control of Deafness (2006) • National Tobacco Control Programme (2007) • National Programme for Prevention and Control of Fluorosis (2008) • National Programme on Prevention and Control of Diabetes, CVD and Stroke (2010) • National Programme for Prevention and Management of Trauma and Burn Injuries (2010) • National Programme for Health Care in Elderly (2010) • National Programme for Palliative Care (2012) • RMNCH+A strategy (2013) • National programme on AMR containment (2013) • India Newborn Action Plan (2014) • National Oral Health Programme (2014) • Ayushman Bharat (2018) • National Programme on Prevention and Control of Viral Hepatitis (2018) • National Rabies Control Programme (2018) • National Sickle Cell Anaemia Elimination Mission (2023)	

Ministry of Health and Family Welfare Budget

Ministry of Health and Family welfare has two Departments. 1) Department of Health and Family welfare 2) Department of Health Research. Apart from that, Ministry of Health and Family welfare has two attached offices; Directorate General of Health Services (DGHS) and National Health Authority (NHA).

Ministry's budget allocation has been increasing gradually to focus on strengthening existing health systems and establishing new health facilities.

Fig 4 - MoHFW Budget 2023-2024⁵

For 2023-24, 96.7% of the budget was allotted to Department of Health and Family from the total budget of **INR 89,155 crore**

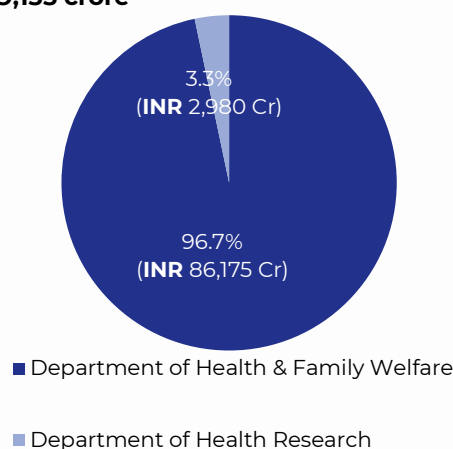
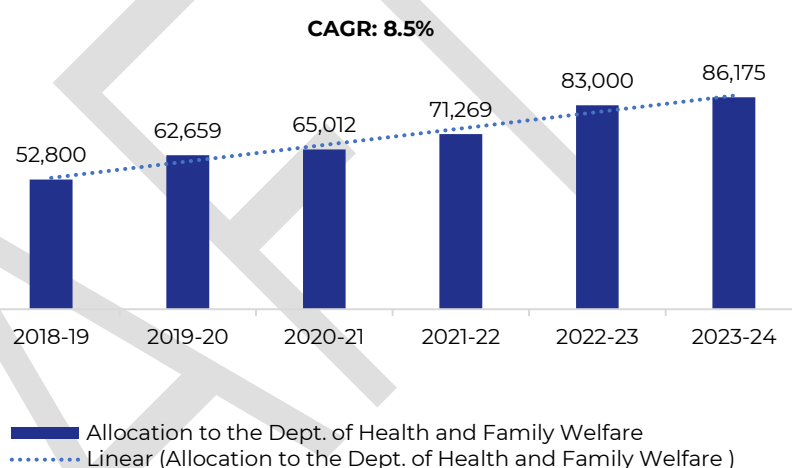
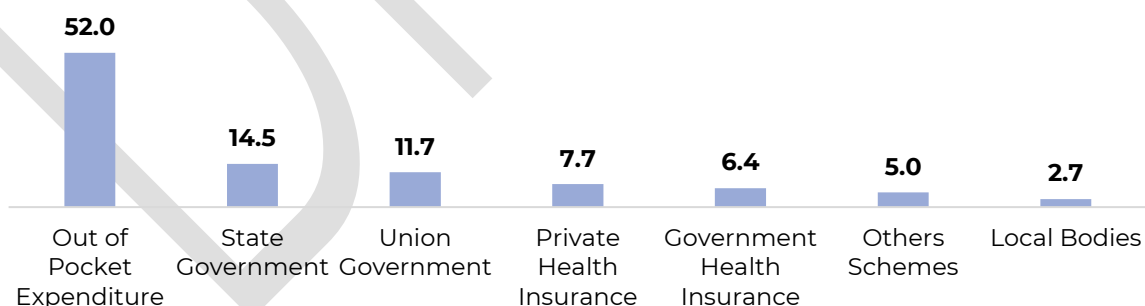


Fig 5 - Budget allocation to the Department of Health and Family Welfare over Years (in INR, Cr)^{5,6}



Healthcare Spending⁷

Fig 6 - % Distribution of Current Health Expenditure (2019-20) according to Healthcare Financing Schemes



*Other schemes include Non-Profit Institutions Serving Households (NPISH) (1.72%), Residents Foreign Agencies Schemes (0.17%), and Enterprise's financing schemes (3.07%).

⁵ Ministry of Health and Family Welfare. (2023). *Detailed Demand for Grants 2023-24*.

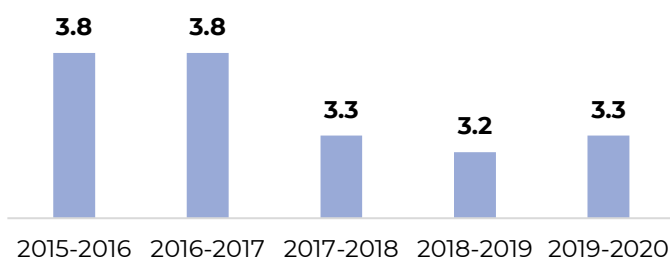
⁶ Ministry of Health and Family Welfare. (2022). *Detailed Demand for Grants 2021-22*.

⁷ Ministry of Health and Family Welfare. (2023). *National Health Accounts Estimates for India 2019-20*.

Fig 7 - Trends In Broad Health Financing And Macroeconomic Indicators

Sr. No.	Indicators	2018-2019	2019-2020	% Change
		(INR in crores)		
1	Gross Domestic Product (GDP)	1,88,99,668	2,00,74,856	6%
2	General Government Expenditure	50,40,707	54,10,887	7%
3	Total Health Expenditure (THE)	5,96,440	6,55,822	10%
4	Government Health Expenditure	2,42,219	2,71,544	12%

Fig 8 - Total Health Expenditure (THE) as a % of Gross Domestic Product (GDP)



In the fiscal year 2019-20, India's Total Health Expenditure was INR 6,55,822 crores, which accounts for 3.27% of the GDP and amounts to INR 4,863 per capita. THE encompasses both current and capital expenses, encompassing funds expended by government and private entities, including contributions from external donors.

Fig 9 - Split of Total Health Expenditure (THE)

- Government Health Expenditure (GHE), which includes capital expenses, stands at INR 2,71,544 crores, constituting 41.41% of THE.
- Households' Out of Pocket Expenditure on healthcare (OOPE) amounts to INR 3,08,727 crores, making up 47.07% of THE.

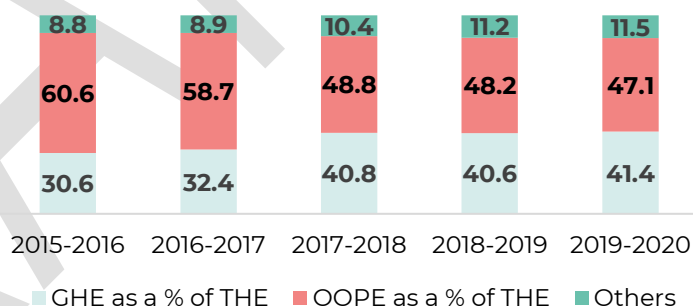
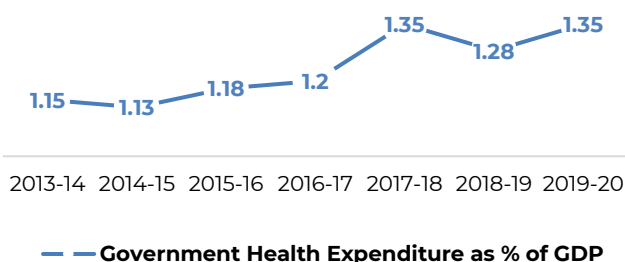


Fig 10 - Government Health Expenditure as % of GDP 2013-2014 to 2019-2020



Government Health Expenditure (GHE)

- GHE has shown a positive trend, rising from 1.15% in 2013-2014 to 1.35% in 2017-2018 and then again rising to 1.35 in 2019-2020.
- Government Health Expenditure includes all schemes funded and managed by Union, State, and Local Governments, including Governmental organisations and donors in case funds are channelled through Government organisations.



Trends in Health Sector

India's healthcare sector comprises hospital infrastructure, medical devices and equipment, health insurance, clinical trials, telemedicine, and medical tourism.



1. Healthcare delivery and infrastructure: Hospitals include government (healthcare centres, district hospitals, general hospitals) and private hospitals (nursing homes, mid-tier and top-tier private hospitals). Health Infrastructure consists of doctors, nurses, hospitals, and various paramedical professionals, equipment and beds needed in the hospitals and a well-developed pharmaceutical industry.



2. Health Insurance Coverage: Health insurance includes health insurance and medical reimbursement facilities, covering an individual's hospitalisation expenses incurred due to sickness.



3. Human Resources for Health (HRH): The healthcare workforce in India is a diverse group of professionals who provide healthcare services to the population. The workforce includes doctors, nurses, pharmacists, dentists, allied health professionals, and traditional medicine practitioners.



4. Pharmaceuticals and Medical devices: Major segments of pharmaceutical industry include generic drugs, OTC, medicines, bulk drugs, vaccines, contract research and manufacturing, biosimilars, and biologics. Medical equipment comprises surgical, dental, orthopaedic, etc. Diagnostics sector includes analytical and diagnostic services. The current market size of the medical devices industry in India is estimated at USD 11 billion.



5. Biotechnology: India is among the top 12 destinations for biotechnology worldwide and 3rd largest destination for biotechnology in Asia Pacific. In 2023, India's Biotechnology industry has crossed an estimated USD 92 billion, growing 15% from the previous year.



6. AYUSH: AYUSH is derived from a Sanskrit word "ayusmanbhava" for long life and a term for various traditional and non-conventional systems of health care and healing, such as Ayurveda, Yoga, Unani, Naturopathy, Siddha, Sowa Rigpa, Homoeopathy, etc. AYUSH is a holistic and integrative approach to physical, mental, social, and spiritual well-being that is honoured as part of religion and has been serving humanity for a long time.



7. Digital Health: Digital health interventions strengthen outcomes of every health service delivery programme in the country. India has driven its healthcare capabilities with the launch of the Ayushman Bharat Digital Mission. India is shifting its traditional method of doctor-patient interaction to digitally enabled remote consultations as it is having enormous potential in education, training, and management in this sector.

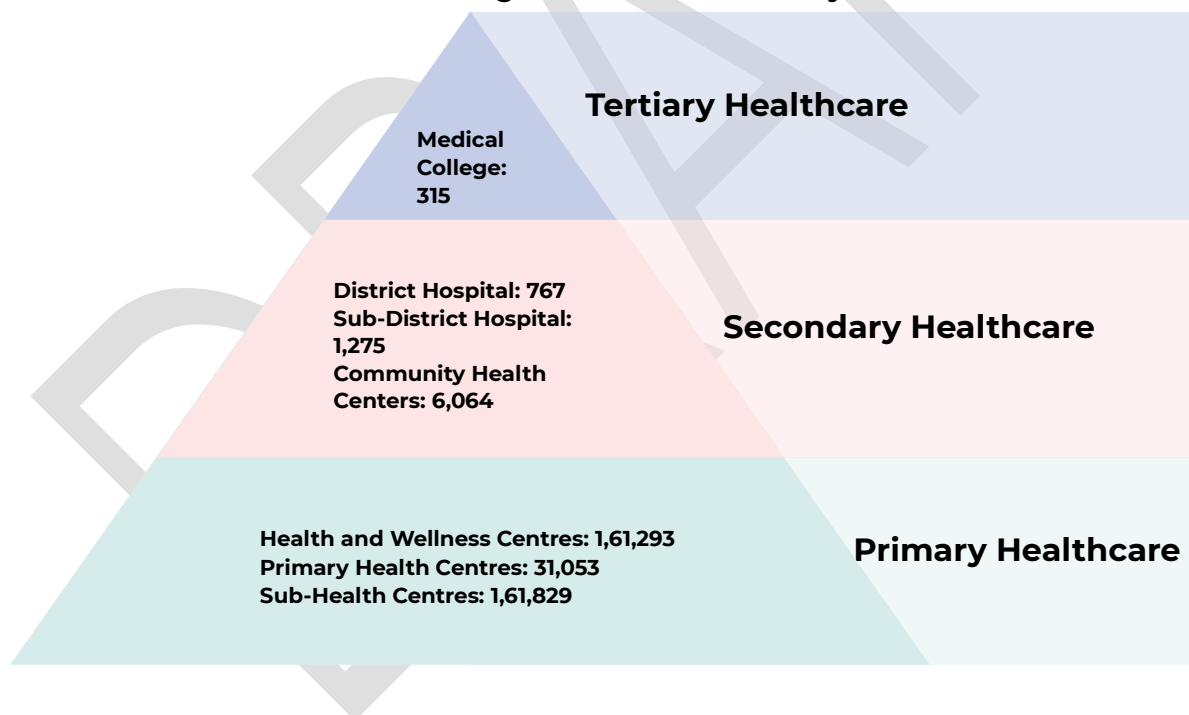


8. Medical Value Travel: Medical tourism or Medical Value Travel defined as travel for the purpose of maintaining, improving, or restoring health through medical intervention. It includes activities related to travel and hosting foreign tourists to maintain, improve, and restore health through medical interventions such as treatment (traditional and non-traditional), wellness, and rejuvenation.

1. Healthcare Delivery and Infrastructure

Public healthcare is delivered through a three-tiered model with a network of sub-centres, Health and Wellness Centres (HWCs), and Primary Health Centres (PHCs) at the base, and specialised medical hospitals at the apex of the pyramid.

Fig 11 - Healthcare Delivery Model⁸



⁸ MoHFW, (2022). Rural Health Statistics 2021-22.



Primary Health Care

1. Sub-Health Centres⁸

2. Primary Health Centres⁸

3. Health and Wellness Centres⁸

Primary healthcare serves as the initial point of contact between individuals and the healthcare system, delivering "essential" healthcare services. The majority of common health issues and concerns can be effectively addressed at this primary level of care, which is situated closest to the community. In the Indian context, primary healthcare is offered through primary health centres, their subcentres and Health and Wellness Centres with active community involvement.



Secondary Health Care

1. Community Health Centre⁸

2. Sub District⁸

3. District Hospitals⁸

At this level, more complex problems are dealt with. This care comprises essentially curative services and is provided by the district hospitals and community health centres. CHCs serve as the first referral level in the health system.



Tertiary Health Care

1. Medical College and Hospitals⁸

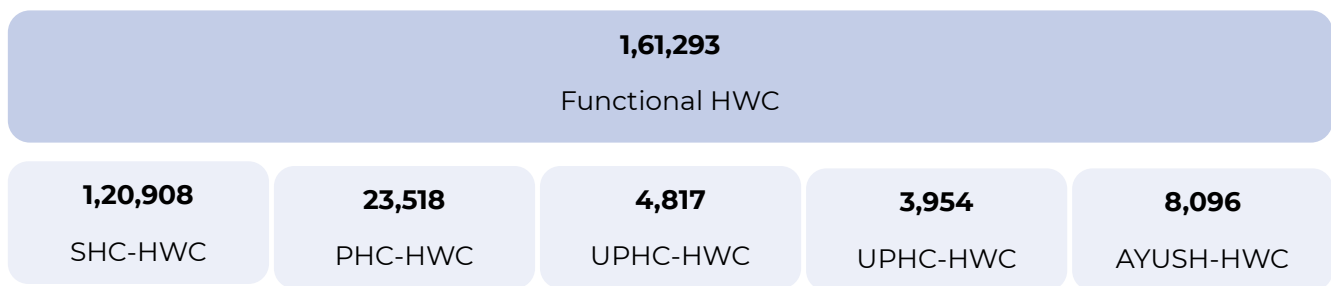
At this stage, super-specialised healthcare is available and administered by regional or central institutions. These institutions not only deliver highly specialised medical treatment but also offer expertise in planning, management, and education for specialised healthcare professionals. Moreover, the tertiary level of healthcare supports and augments the efforts undertaken at the primary level.

Fig 12 - Number of Health Facilities (2022)⁸

Sub Centres	161,829
Primary Health Centres	31,053
Community Health Centres	6,064
Sub Divisional/ District Hospitals	1,275
District Hospitals	767

- Between 2005 and 2022, number of centres increased by:⁸
 - Sub-centres (SC): 11,909
 - Primary Health centres (PHC): 1,699
 - Community Health centres (CHC): 2,134
- Allopathic doctors number increased by 50.9% at PHCs from 20,308 in 2005 to 30,640 in 2022.⁸
- As of 2022 – 25,383 SCs and 3,833 PHCs are operational in the tribal areas representing India's effort to reach more and more underserved areas.⁸

Ayushman Bharat – Live Health and Wellness Centres Numbers⁹



*Data as on 13th October, 2023

Fig 13 - Shortfall in Healthcare Manpower at SCs and PHCs in % (2021-22)⁸

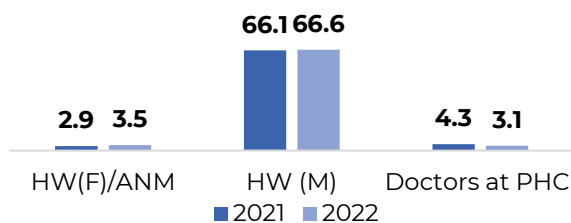
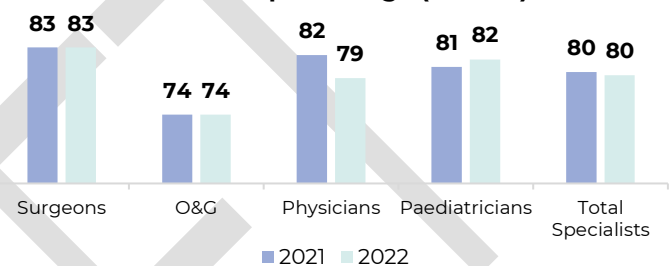


Fig 14 - Shortfall in Healthcare Manpower at CHCs in percentage (2021-22)⁸



- There is a shortfall of 3.1% in allopathic doctors at PHC, of the total requirement at all India levels.⁸
- Overall, 67.8% of the sanctioned posts of specialists at CHCs are vacant, and there is a shortfall of 79.5% specialists at the CHCs as compared to the requirement.⁸

The Indian healthcare industry is becoming more concentrated, with fewer and larger hospitals controlling a greater share of the market. This is being driven by a number of factors, including the COVID-19 pandemic, which has led to people seeking higher-quality healthcare, and the increasing demand for specialized services. Patients are increasingly moving from smaller nursing homes to larger hospitals, which offer a wider range of services and better-quality care.

The growing demand for healthcare in India is driving further expansion in the hospital industry. Many hospital companies have announced significant plans to add new beds and upgrade their facilities over the next four years. This expansion will add nearly 8,400 new beds to the market, representing a more than 26% increase in capacity from March 2023 levels.¹⁰

HW(F): Health Workers Female, HW(M): Health Workers Male, ANM: Auxiliary Nurse Midwife

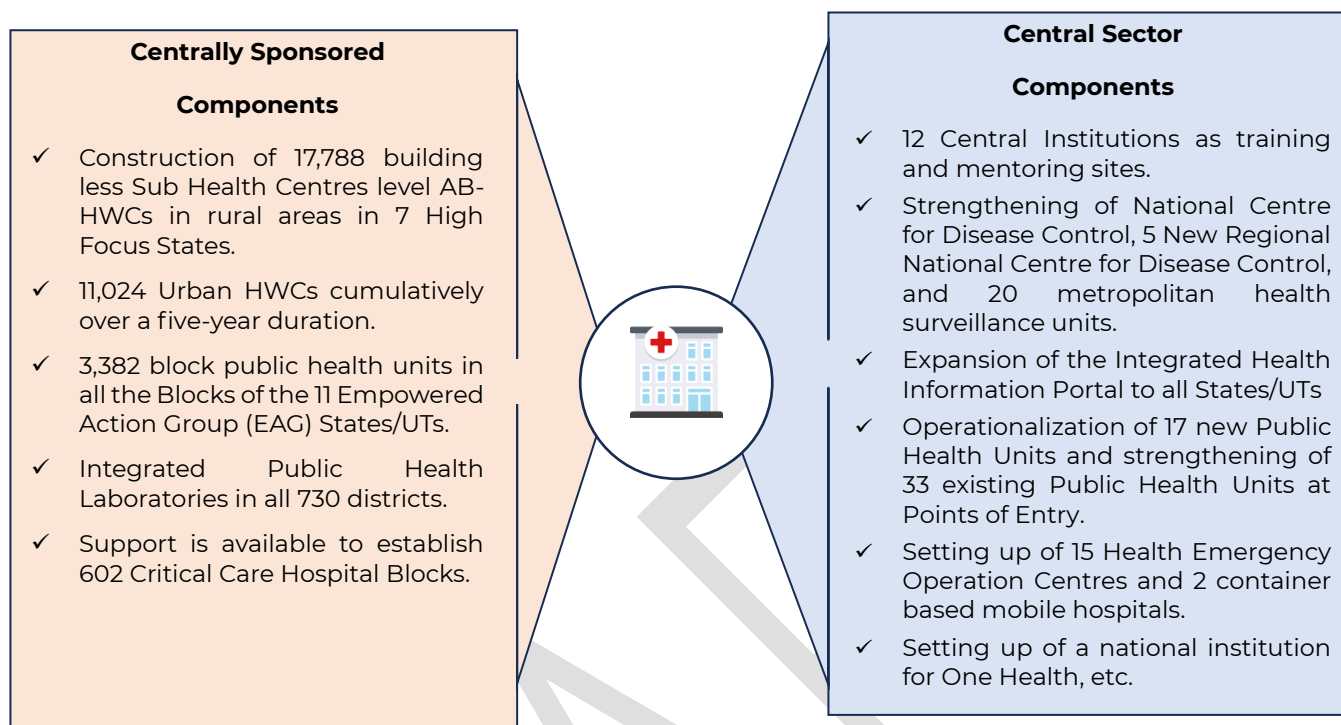
⁹ Health and wellness Centre- National Health Mission. (2023). *Ministry of Health and Family Welfare*. Retrieved on 25 September 2023.

¹⁰ TPCI (Trade Promotion Council of India). (2023). *Indian hospital industry garnering investors' attention*. Retrieved on 25 September 2023.

Initiatives for Healthcare Infrastructure: Ayushman Bharat Programme

1. PM-ABHIM¹¹

Aims to make all districts self-reliant under this scheme in terms of infrastructure facilities and health care delivery.



2. Pradhan Mantri Swasthya Suraksha Yojna (PMSSY)

It was launched in 2006, aiming to establish advanced healthcare infrastructure in underserved regions, focusing on medical education, research, and clinical care. It strives to address regional disparities in the accessibility of affordable and dependable tertiary healthcare services, while also enhancing the resources for high-quality medical education.¹²

- The scheme comprises two main aspects: Establishment of All India Institute of Medical Sciences (AIIMS) and Enhancement of existing Government Medical Colleges/Institutions (GMCIIs).
- For 2023-24, the estimated budget for scheme is INR 3,365 Cr.¹³

Within the schemes, two components exist;



6 AIIMS in Phase-I: Bhopal, Bhubaneswar, Jodhpur, Patna, Raipur, and Rishikesh have fully operational AIIMS and more hospitals are being set up under different phases.¹²

Upgrading Existing State Government Medical colleges and institutions: A total of 75 projects/State government colleges have been approved, out of which 60 projects are already completed.¹²

¹¹ Ministry of Health and Family Welfare. (2022). *Note on PM- Ayushman Bharat Health Infrastructure Mission*.

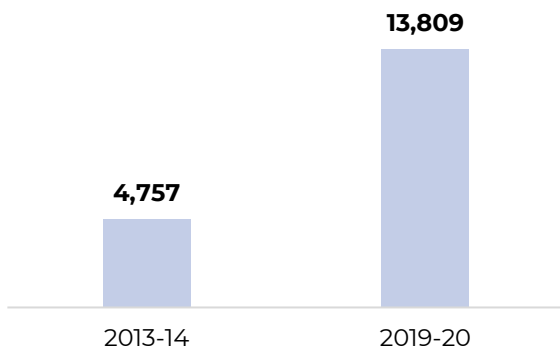
¹² Ministry of Health and Family Welfare. (2023). *Pradhan Mantri Swasthya Suraksha Yojna: About the Scheme*. Retrieved on 25 September 2023

¹³ India Budget. (2023). *Department of Health and Family Welfare Budget*.

2. Health Insurance Coverage

Majority of health expenditure still comes from out of pocket. Health care financing schemes include Private Health Insurance (7.72%), Union Government (11.74%), Government Health Insurance (6.37%), State Government (14.52%), Local Bodies, 2.69%, and Other Schemes (4.96%). Private Indian health insurance industry reported a growth of 26.3% in FY 2022 from previous year.¹⁴

Fig 15 - Government Financed Health Insurance⁷



- External funding has decreased significantly and government financed health insurance expenditure increased by 190% from 2013-2014 to 2019-20.⁷
- Government Health Insurance Schemes include:
 - ✓ Employees' State Insurance Scheme (ESIC)
 - ✓ Central Government Health Scheme (CGHS)
 - ✓ Ex-servicemen Contributory Health Scheme (ECHS)
 - ✓ Government-based voluntary insurance schemes like PMJAY, RSBY, state specific government health insurance schemes etc.

Pradhan Mantri Jan Arogya Yojana (PM-JAY)¹⁵

- Pradhan Mantri Janrogya Yojana (PM-JAY) provides health insurance cover of INR 5 lakhs per year to over 10 crore poor and vulnerable families for seeking secondary and tertiary care.
- Under AB-PMJAY, National Health Authority (NHA) has introduced an updated Health Benefit Package for the year 2022 which incorporates 365 new procedures, leading to a total of 1,929 procedures covered under the scheme.
- 26,054 hospitals have been empanelled and 4.25 crore hospital admissions have been authorized under AB-PMJAY as of March 2023.¹⁶

Key Features of PM-JAY¹⁵

- PM-JAY is the world's largest health insurance/ assurance scheme fully financed by the government.
- It provides a cover of INR 5 lakhs per family per year for secondary and tertiary care hospitalisation across public and private empanelled hospitals in India.
- Over 12 crore poor and vulnerable entitled families (approximately 55 crore beneficiaries) are eligible for these benefits.
- PM-JAY provides cashless access to health care services for the beneficiary at the point of service, that is, the hospital.
- PM-JAY envisions to help mitigate catastrophic expenditure on medical treatment which pushes nearly 6 crore Indians into poverty each year.
- It covers up to 3 days of pre-hospitalisation and 15 days of post-hospitalisation expenses such as diagnostics and medicines.
- There is no restriction on the family size, age or gender.
- All pre-existing conditions are covered from day one.
- Benefits of the scheme are portable across the country i.e.; a beneficiary can visit any empanelled public or private hospital in India to avail cashless treatment.
- Services include approximately 1,929 procedures covering all the costs related to treatment, including but not limited to drugs, supplies, diagnostic services, physician's fees, room charges, surgeon charges,

¹⁴ Invest India (2023). Snapshot Insuring India. Retrieved on 11 October, 2023.

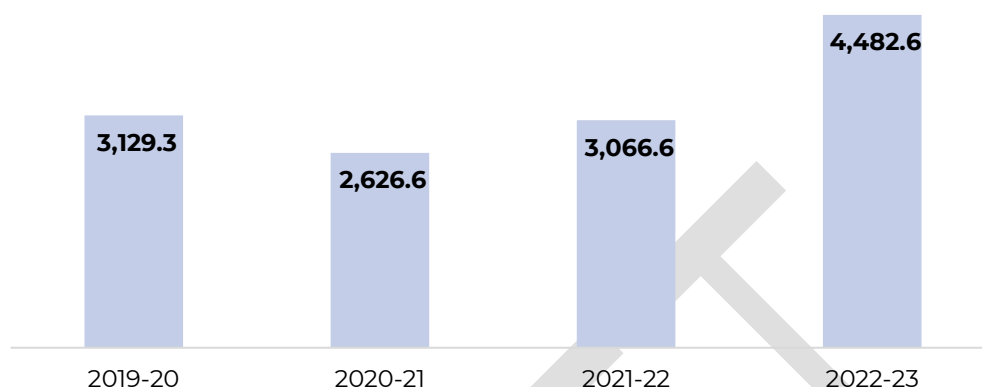
¹⁵ National Health Authority: About Pradhan Mantri Jan Arogya Yojana (PMJAY).

¹⁶ Ministry of Health and Family Welfare (2022). MoHFW Annual Report 2022-23.

OT and ICU charges etc.

- Public hospitals are reimbursed for the healthcare services at par with the private hospitals.

Fig 16 - PM-JAY Fund Released in INR Cr¹⁶



3. Human Resources for Health (HRH)

Healthcare sector in India became the fifth largest employer in 2015, employing 0.47 Cr people directly.³² During the 3rd round of the Quarterly Employment Survey (2021), there was a notable increase in the number of workers in the health sector. The proportion of total workers employed rose from 8.4% in April 2021 to 10.4% in October 2021.¹⁷



Medical Doctors (2019)

0.93 per 1,000²



Nurses and Midwives (2019)

2.39 per 1,000
population²



Pharmacists (2019)

0.88 per 1,000
population²



Dentists (2019)

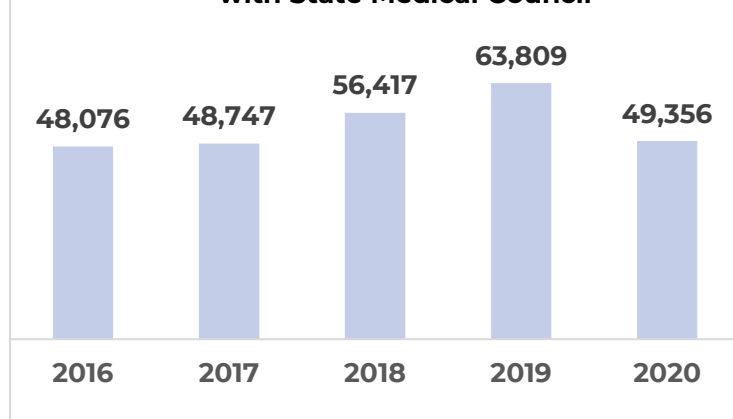
0.2 per 1,000
population²

- Overall health worker density in India per 1,000 population increased from 2.9 health workers per 1,000 population in 2016 to 4.4 per 1,000 population in 2019. However, service delivery is still affected by inadequate strength of healthcare workers.²
- Doctor patient ratio has increased in the last three years. However, unequal distribution of healthcare workers still exists between urban and rural areas.²

Inadequate health workforce is a major source of inefficiency in delivering government health-care services in India. Assessment of trends in health worker density from 1991 to 2019 suggests that there is a chronic shortage of physicians, with a density rate of 0.93 per 1,000 persons in 2019. Hence, several initiatives to strengthen the health workforce in India have been undertaken, particularly under the NRHM/NHM, including the recruitment of health workers on a contractual basis to strengthen the health workforce in rural areas. In addition, financial incentives, such as honorariums for specialists in rural areas to conduct C-sections, and allowances for service in difficult areas, as well as non-financial incentives like preferential admission to post-graduate programmes for health workers serving in difficult areas, and improved accommodation are offered to strengthen the rural health workforce²

¹⁷ Ministry of Labour and Employment. (2022). *Labour and Employment Statistics 2022*.

Fig 17 - Year wise Doctors Registration with State Medical Council¹⁸



There are 593 medical colleges training students, with approximately 88,170 MBBS admissions available (National Medical Commission, 2021). However, the distribution of medical colleges is highly skewed, with Telangana, Andhra Pradesh, Tamil Nadu, Kerala and Karnataka in the south and Maharashtra in the west accounting for nearly 54% of total MBBS seats, despite having less than 30% of the country's population. The number of medical colleges and training slots/seats has been steadily rising and are reflected in the growing numbers of doctors registered with state medical councils²

4. Pharmaceuticals and Medical Devices

Pharmaceuticals

The Indian pharmaceutical industry plays a significant role globally, supplying affordable and low-cost generic drugs to millions of people worldwide thus impacting health outcomes on a global scale.¹⁹ The Indian pharmaceutical industry **ranks third globally** in terms of volume and fourteenth in terms of value.²⁰ India is the only country that has the highest number of US Food and Drug Administration (USFDA) approved pharmaceutical plants, outside of US. In addition to this, India also has a substantial number of plants that adhere to WHO-GMP standards along with those that have received approval from regulatory authorities in various other nations. Indian medicines are preferred worldwide due to their low cost and high quality, earning it the title of "pharmacy of the world."



3rd

rank globally in pharmaceutical production by volume¹⁹



60%

of global vaccine production¹⁹



INR 3,44,125 Cr

annual turnover of the pharma sector in 2021-22¹⁹

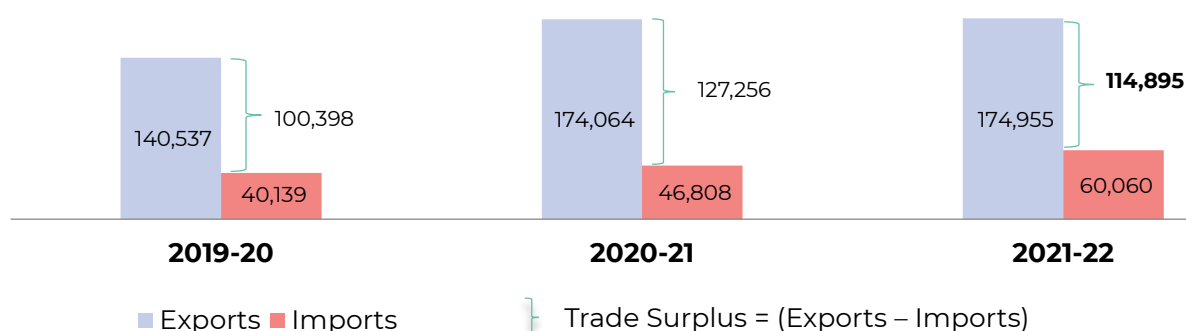
- Major segments of Indian Pharmaceutical Industry include **generic drugs, OTC medicines, bulk drugs, vaccines, contract research and manufacturing, biosimilars and biologics**.¹⁹
- The Indian pharmaceutical industry is known for its generic medicines and low-cost vaccines. Globally, India is the largest supplier of generics and is known for its drugs and vaccines.¹⁶ Indian pharmaceutical industry is currently valued at **INR 3.4 Lakh Crore** and Indian pharma products are exported to **200+ countries**^{19,20}. India supplies over 50% of Africa's requirement for generics, **40%** of generic demand in the US, and **25%** of all medicine in the UK.¹⁹
- India also has a strong presence in the active pharmaceutical ingredient (API) industry, with 500 API manufacturers and contributes **8% in the global API industry**.¹⁹
- Furthermore, India is the world's largest provider of generic medicines, manufacturing about **60,000 different generic brands across 60 therapeutic**.¹⁹

¹⁸ Central Bureau of Health Intelligence. (2022). National Health Profile 2022 17th Issue.

¹⁹ Department of Pharmaceuticals. (2023). Annual Report.

²⁰ Invest India. (2022). Pharmaceuticals.

**Fig 18 - Annual Import-Export
(in ₹ Cr)¹⁹**



Medical Devices

The Medical Devices sector has been recognized as a sunrise sector under the Make in India campaign. Backed by the growing medical needs of the country, Indian medical device market is expected to quadruple by 2030.²¹



INR 90,000 Cr

size of the Indian medical devices market in 2022²¹



16.4%

expected growth rate by 2030²¹



1.7%

market share in the global market²²



4th

largest Asian medical devices market after Japan, China, and South Korea²²

- Over the past five years (2017-2018 to 2021-2022), the medical device export sector has witnessed a CAGR of **9.4%**.¹⁹
- In 2021-22, medical devices worth **INR 23,982.3 crore** were exported.¹⁹
- The current market size of the medical devices industry in India is estimated at **USD 11 billion**. India is the fastest-growing medical devices market amongst the emerging markets.²¹
- FDI up to **100%** under the automatic route is permitted for the manufacturing of medical devices.¹⁹
- Market size expected to reach **USD 50 billion** by 2030.²¹
- The export of medical devices sector has been growing at a CAGR of **9.37%** over the last 5 years. India's expected export of medical devices will reach ~ **USD 10 billion** by 2025.²¹

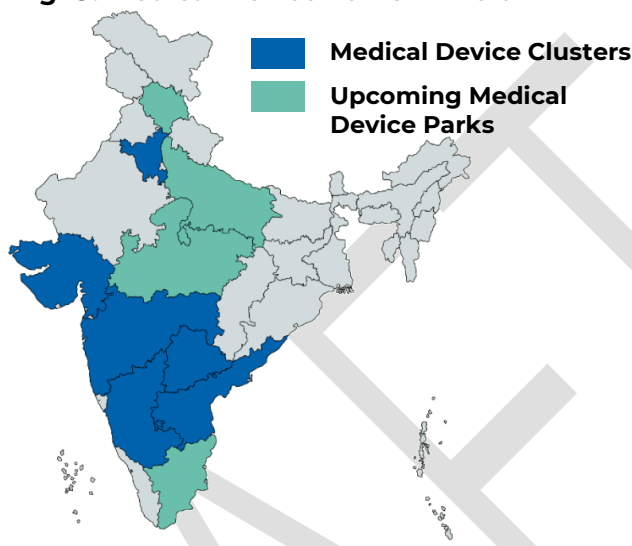
²¹ Invest India. (n.d.). *Medical Devices*. Retrieved 12 September 2023.

²² NITI Aayog. (2021). *Investment Opportunities in India's Healthcare Sector*.

National Medical Device Policy, 2023²³

- The vision of the policy is to pursue an accelerated growth trajectory with a patient-centric focus, aiming to establish India as a global leader in medical device manufacturing and innovation.
- The goal is to attain a 10-12% share of the expanding global market within the next 25 years.
- Strategies: Regulatory Streamlining, Enabling Infrastructure, Facilitating R&D and Innovation, Attracting Investments in the Sector, Human Resources Development and Brand Positioning and Awareness Creation.

Fig 19: Medical Device Parks in India¹⁹



5. Biotechnology

Biotechnology is the use of biological knowledge and techniques to develop new products and services using molecular, cellular and genetic methods. Biotechnology is known as a sunrise sector that is driving the growth of the economy in India. The Department of Biotechnology (DBT), under the Ministry of Science and Technology, aims to make India a global leader in biotechnology by supporting research, innovation, and entrepreneurship with an aim to achieve National Development Programmes (NDPs) and Sustainable Development Goals (SDGs). India is ranked 12th in the biotechnology sector, 3rd in the Asia-Pacific region, and the world's largest vaccine manufacturer.²⁴



Fig 20 - Indian Biotech Industry Valuation (in INR L Cr)²⁵

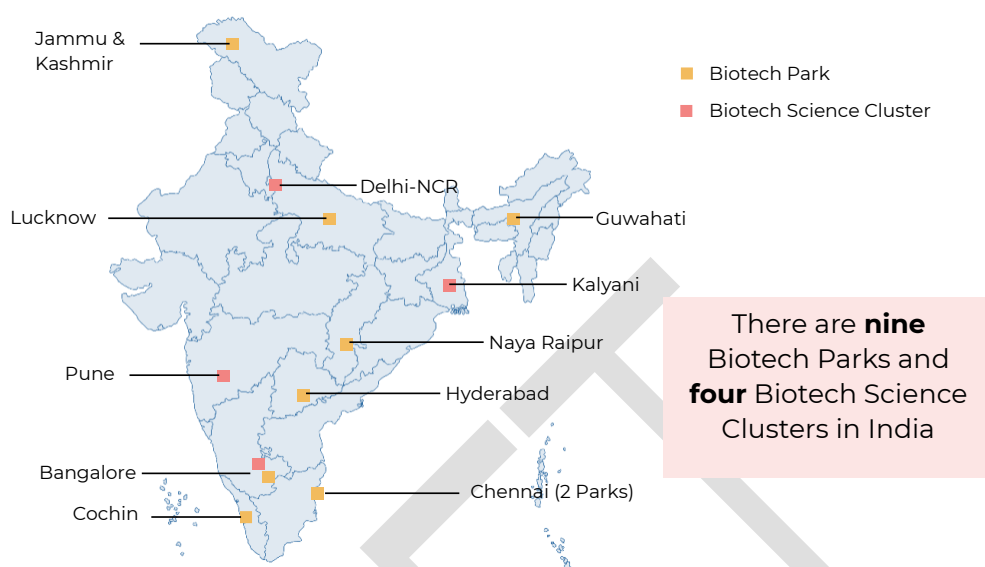


²³ Department of Pharmaceuticals. (2023). National Medical Device Policy 2023.

²⁴ Department of Biotechnology. (2023). Annual Report 2022-2023

²⁵ Department of Biotechnology. (2021). National Biotechnology Development Strategy (2021-2025)

Fig 21 - Biotech Parks and Clusters in India²⁴



Research Landscape in Biotechnology

Government of India is committed to promoting biotech-based drug development under the "Make in India" campaign. Multiple reports have summarized initiatives introduced by the Department of Biotechnology, Biotechnology Industry Research Assistance Council and the government of India to support the vision of making India a hub for biotechnology-based innovation and research.²⁶

Some of the prominent areas of Biotech research include: -



Vaccine



Oncology
research



Stem Cells
and
Regenerative
Medicine



Genome Editing
Applications in
Healthcare



Biomedical
Engineering

6. AYUSH - Alternative Systems of Medicines²⁷

AYUSH is an acronym for **Ayurveda, Yoga and Naturopathy, Unani, Siddha, Sowa Rigpa and Homoeopathy**. Ayurveda is the oldest system with a documented history of its practice for more than 5,000 years whereas Homoeopathy is in practice in India for around 100 years. About **80-90%** of population in India is reported to use Ayurveda and other traditional systems of medicine to meet their primary health care needs.

²⁶ BIRAC. (2019). *India: The emerging hub for biologics and biosimilars*.

²⁷ Ministry of Ayush. (2023). *Annual Report 2022-23*.

National Ayush Mission (NAM)



NAM aims at the improvement in quality of AYUSH Education through upgrading existing AYUSH Educational Institutions and establishment of new Ayush Colleges in the States where availability of Ayush teaching institutions is inadequate in Government Sector.

• Public Health Programmes

Building upon the past experiences and fine tuning the need assessment cum need response strategies to widen the coverage of AYUSH services, the Ministry of Ayush incorporated 8 dedicated Ayush Public Health Programmes in NAM this year. These programmes are designed to cover health issues of all major vulnerable groups thus making a continuum of Ayush services for all sections of the society. The fruition of these programmes' delivery would, ultimately, reach every corner of the country.

Major Achievements:



Establishment of WHO- Global Centre for Traditional Medicine (WHO-GCTM): Establishment of WHO Global Centre for Traditional Medicine was for improving global health and achieving Universal Health Coverage



Conducive ecosystem for start-ups and MSMEs: A conducive ecosystem for start-ups and MSMEs is emerging in Ayush sector, spurring innovation at a massive scale in the currently developing Ayush sector. At present there are about 53,023 MSMEs (2022 Data) in Ayush as per MSME's Udyam Portal. The All India Institute of Ayurveda (AIIA) an autonomous body under Ministry of Ayush has set up an incubation centre i.e. AIIA-ICAINE (Incubation Centre for Innovation and Entrepreneurship) to support new start-ups in the Ayush sector.



Advanced R&D through Ayush-based fundamentals: Under Centre of Excellence of Ministry of Ayush at Institute of Genomics and Integrative Biology (IGIB) CSIR, relation of Ayurveda Prakriti is made with the Genome sequence, making it a landmark study towards personalised preventive and predictive medicine, they are also getting promising outcomes on Gut Microbiota and working on advance biology of metabolomics, Proteomics etc. to shape the future of healthcare sector.

R&D initiatives and COVID-19 management



150 research studies were done in Ayush systems and based on R&D outcomes one Ayurveda and one Siddha Medicine was successfully repurposed for management of COVID-19 and provided to the public all across the country at large. Total of 63 Research Publications are done in reputed Journals, 33 are available in preprints and 40 are in different levels of publications. Massive response from public toward Ayush during COVID-19, as per a study published in PubMed indexed journal 14 JMIRx Med (impact factor 4.67). In 2021, shows that, a mobile application 'Ayush Sanjeevani' based impact assessment study undertaken by the Ministry of Ayush, to study the effect of Ayush Advisory and other Ayush interventions and practices during COVID-19. A total of 1.33 crore data was recorded and data of 723,459 respondents were analysed, it shows that 85.1% of the respondents reported use of Ayush measures and 89.8% respondents agreed to have benefitted from the practice of Ayush advisory.

Ayush medicines included in National Clinical Management Programme for COVID-19 management



Ayush medicine provided to front line workers and helped reduced incidence of COVID-19 among them: As per a study published in Frontiers in Public Health (impact factor 5.99) percentage of COVID-19 IgG positivity was significantly lower in the Delhi Police group (17.5%) who were provided Ayu-Raksha Kit by All India Institute of Ayurveda as compared to the control group, General Delhi Population (39.4 %, $p = 0.003$), indicating the lower risk (55.6%) of COVID-19 infection in the Delhi Police group, further there was decreased incidence (5.05%) and reduced mortality percentage (0.44%) of COVID-19 among Delhi police officers during peak times of the pandemic. Around 7 lakh Ayush doctors and Ayush hospitals offered services for COVID management.

7. Digital Health

In recent years, India has witnessed a remarkable evolution and progress in the field of digital health. This document explores the transformative impact of digital health technologies on healthcare delivery, patient outcomes, and the healthcare ecosystem as a whole with respect to India and also delves into the key highlights of various initiatives by the Government of India to boost and encourage the growth of digital health in the country.

Key enablers of digital health: India has seen a significant transition in internet usage, increasing from 5% in 2009 to 46% in 2021.²⁸ Moreover, there has been a remarkable 200% surge in internet penetration in rural India between 2015 and 2021, underscoring substantial progress towards a digitally connected nation.²⁹ In shaping the future of digital health, three pivotal factors - digital health innovations, Jan Dhan Aadhaar Mobile (JAM) trinity, and increased bank account penetration - are playing a transformative role, driving accessibility and efficiency in healthcare services.

Trends in digital health:

Ayushman Bharat Digital Mission (ABDM): The Ayushman Bharat Digital Mission (ABDM) is a national digital health ecosystem that was launched on March 2020, which connects the digital health solutions of healthcare providers across the country and provides a wide range of data, information, and infrastructure services to support universal health coverage.

Building Blocks³⁰

- **ABHA Number:** Patients are allotted Ayushman Bharat Health Account (ABHA) number to uniquely identify medical records and ensure that the created medical records are issued to the correct individual.
- **Healthcare Professionals Registry (HPR):** A comprehensive repository of all healthcare professionals involved in delivery of healthcare services. This registry will enable them to get connected to India's digital health ecosystem.
- **ABHA Mobile App (PHR):** An electronic record of health-related information on an individual that conforms to nationally recognised interoperability standards and that can be drawn from multiple sources while being managed, shared, and controlled by the individual.
- **Health Facility Registry (HFR):** A comprehensive repository of health facilities of the nation across different systems of medicine (both private and public facilities).

Adoption of Telehealth /Telemedicine: Telemedicine in India has gained significant traction and is experiencing a notable rise in popularity and adoption. Major hospitals like Apollo, AIIMS, Narayana Hrudayalaya have adopted telemedicine services and entered into a number of Private-Public Partnerships.³¹



Future
Growth

- Telemedicine market in India is expected to reach a valuation of INR 44,416 crores by 2025, growing at a CAGR of 31%.³²
- Telemedicine initiatives are being undertaken by several private sector companies such as Practo, Tata 1mg, and Apollo 24x7. Among these, Pharmeasy has achieved a valuation of INR 1,231 Crores in the field of telemedicine.³²

²⁸ World Bank. (n.d.) Individuals using the Internet (% of population) – India. Retrieved 19 September 2023.

²⁹ India budget. (2023). Economic Survey 2022-2023.

³⁰ National Health Authority. (2023). Ayushman Bharat Health Mission. Retrieved on 1 September.

³¹ Shetty KP (2022). A Study on the Transformation of the Indian Healthcare Sector during the COVID-19 Pandemic. International Journal for Research in Applied Science and Engineering Technology.

³² InvestIndia. (n.d.). The Indian Unicorn Landscape. Retrieved 8 September 2023.

³³ Centre for Development of Advanced Computing. (2023). Sanjeevani Landmark Milestone. Retrieved 17 September 2023.

³⁴ Sehat. (2021). SeHAT. Retrieved 17 September 2023.

Major Government Initiatives



eSanjeevani: The world's largest recorded telemedicine implementation functioned through two models: first, a patient-to-doctor telemedicine approach (eSanjeevani OPD), and second, an assisted telemedicine (eSanjeevani Ayushman Arogya Mandir) focused on delivering primary healthcare services. This has reached the landmark milestone of providing telemedicine services to more than 10 Crores patients.³³



SeHAT: e-Health Assistance and Tele-consultation Service is a tri-services teleconsultation service for the Armed Forces of India. Presently, it is designed to fulfil the needs of general OPD consultation. However, there are plans to expand it to include specialist OPD consultation in the future.³⁴

Key Digital Health Initiatives in India

The introduction of multiple digital initiatives in India has led to the creation of a broader spectrum of digital services within the country, which can be grouped as portals and apps as described below:



National Health portal



Ni-Kshay



mDiabetes



Integrated Health Information Platform (IHIP)



Reproductive and Child Health (RCH) Portal



SUGAM



Health Management Information System



Intensified Mission Indradhanush (IMI) Portal



CoWIN Portal:

CoWIN (Covid Vaccine Intelligence Network)



eVIN



National Cold Chain Management Information System (NCCMIS)

8. Medical Value Travel³⁵

The Indian medical value travel (MVT) market is worth INR 0.4 lakh crore and is expected to grow to INR 1.06 lakh crore by 2026.³⁶ Medical tourism is now commonly known as "Medical Value Travel," because it encompasses both patients' healthcare-seeking behavior and the broader economic impact of such travel on the country that hosts them.³⁷ Over the last decade, India has built a reputation for providing high-quality medical care at low costs to medical tourists from all over the world, and it has now emerged as a global leader in the medical tourism sector.³⁷ In terms of market size, the leading countries for wellness tourism are the United States, Germany, China, France, Japan, Austria, and India.³⁸

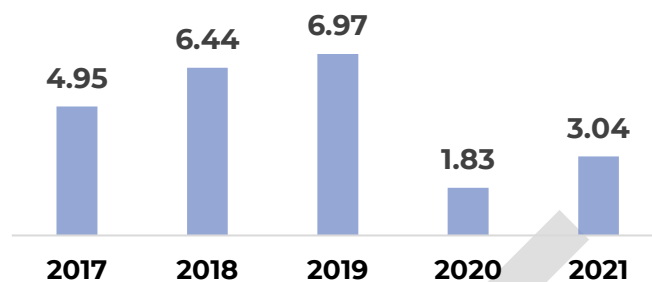
³⁵ Ministry of Tourism. (2022). National Strategy and Roadmap for Medical and Wellness Tourism.

³⁶ Ministry of Tourism. (2021). Status of Medical Tourism in India

³⁷ Invest India. (n.d.). Medical Value Travel. Retrieved 12 September 2023

³⁸ Ministry of Tourism. (2023). Policy for emerging medical tourism.

Fig 22 - Number of Foreign Tourist Arrivals in India on Medical Visa 2017-2021 in Lakhs³⁷



Key Components³⁷

Medical tourism or Medical Value Travel defined as travel for the purpose of maintaining, improving, or restoring health through medical intervention.

Medical Treatment



Curative treatment may involve cardiac surgery, organ transplantation, hip and knee replacement, and other procedures.

Wellness and Rejuvenation



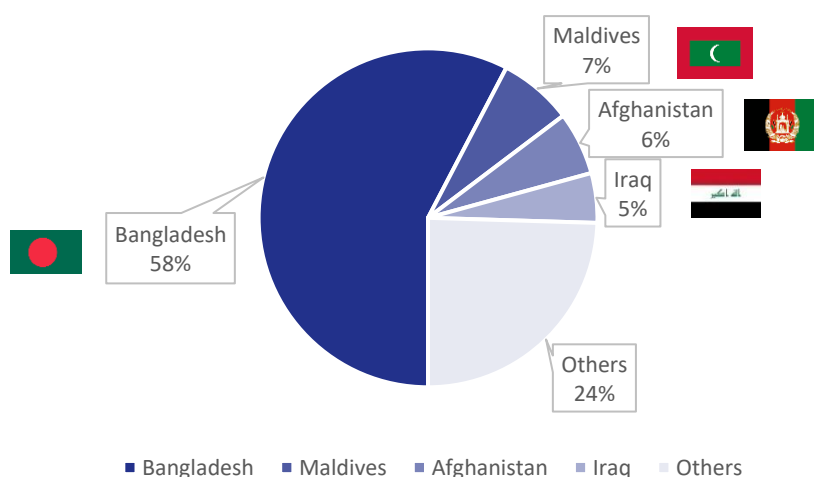
Offerings aimed towards rejuvenation or aesthetic purposes, such as stress treatment, spas, and so on.

Cures through Traditional System of Medicines



Access to traditional medical systems, such as India's AYUSH (Ayurveda, Yoga and Naturopathy, Unani, Siddha, Sowa Rigpa, and Homoeopathy).

Fig 23 - Percentage Distribution of Foreign Tourists' Arrivals from Leading Countries for Medical Purpose During 2020³⁹



- India's medical visitors have historically come primarily from Afghanistan, Pakistan, Oman, Bangladesh, Maldives, Nigeria, Kenya and Iraq.
- Bangladesh contributes to the highest number of tourists arriving for medical purposes every year.
- In 2022, 21.2% of Foreign Tourist Arrival (FTA) in India was for medical purposes.

³⁹ Ministry of Tourism. (2022). *India Tourism Statistics 2022*.

Fig 24 - Medical Procedures in India which cost lower than Major Destinations for Medical Value Travel³⁷

India offers world class treatment at savings of approximately 65-90% compared to the USA. Some of the popular Medical procedures for MVT patients include:

	Heart Bypass		Hip Replacement		Gastric Sleeve
	Heart Valve Replacement		Hip Resurfacing		Gastric Bypass
	Dental Implants		Knee Replacement		Breast Implant
	Lasik		Abdominoplasty		IVF Treatment

Table 1 - Prices of Common Medical Procedures Across Destinations of Medical Value Travel (USD)²²

Medical Procedure	India	Thailand	Malaysia	Singapore	Turkey	South Korea
Heart Bypass	7,900	15,000	12,100	17,200	13,900	26,000
Heart Valve Replacement	9,500	17,200	13,500	16,900	17,200	39,990
Hip Replacement	7,200	17,000	8,000	13,900	13,900	21,000
Hip Resurfacing	9,700	13,500	12,500	16,350	10,100	19,500
Knee Replacement	6,600	14,000	7,700	16,000	10,400	17,500

Low Cost	Medium Cost	High Cost

Opportunities and Challenges

A. Opportunities



Favourable Investment Policies²²

- 100% Foreign Direct Investments through automatic route are allowed for hospitals, healthcare, and medical devices.
- These investments can also support training and capacity-building programs for local



'Heal in India' Attracting More Value to Indian Medicines⁴⁰

- Indian healthcare ecosystem provides treatment across modern medicine, ayurveda, yoga, and other traditional healthcare methods.
- It offers tertiary and quaternary health services, treatment for NCDs, and comprehensive rehabilitation in all major medical specialties, including cardiac care, orthopaedics, etc



Increased Inter Sectoral Convergence⁴¹

- Inter sectoral convergence was initiated under National Urban Health Mission (NUHM) with an aim to improve the effectiveness and utilization of all national health programmes.
- Such practices can be scaled across different levels.



Improving Maternal and Child Health

- Maternal Mortality Ratio in India stood at 97 per lakh live births (2018-2020), 6 points improvement over last year (2017-2019).⁴²
- Infant Mortality Rate (IMR) in India had declined from 40.7 per 1,000 live births for NFHS-4 (2015-2016) to 35.2 per 1,000 live births for NFHS-5 (2019-2020).⁴³



Leveraging Technology To Improve Accessibility⁴⁴

- GoI has established a network of telemedicine services like Hub and Spoke e-Sanjeevni, Tele Manas (teleconsultation for mental health), Nikshay (a web-based TB registration portal), etc., which have improved health outcomes.
- Similarly, platforms like Integrated Government Online training (iGOT), SAKSHAM LMIS, Mobile Academy, etc. are being used for upskilling healthcare workers.

⁴⁰ Ministry of Ayush. (n.d.). Heal in India.

⁴¹ NHSRC. (2021). Inter sectoral convergence under NUHM.

⁴² Census India. (2022). Special Bulletin on Maternal Mortality in India 2018-20.

⁴³ DHS Program. (2022). National Family Health Survey (NFHS - 5), 2019-21 India Report. Ministry of Health and Family Welfare.

⁴⁴ MoHFW. (n.d.). E-Health & Telemedicine.

B. Challenges



Limited Access to Public Healthcare⁴⁵

- Around 92% of healthcare visits are to private providers, primarily by urban residents. However, private healthcare in India is expensive, poorly regulated, and varies in quality. It is also unreliable for those with low literacy and unaffordable for individuals with low incomes.



High Out of Pocket expenditure⁷

- It is estimated that around 63 million individuals are pushed into poverty each year due to insufficient financial protection for their healthcare requirements.



Limitations in the Medical Device industry⁴⁶

- The medical devices industry struggles with higher manufacturing costs, about 12%-15% more than competing economies, due to inadequate infrastructure, domestic supply chain limitations, expensive finance and power, power scarcity, limited design capabilities, and insufficient focus on research and development.



Raw Materials related issues in AYUSH

- There is significant gap in demand and supply of raw materials.
- It is reported that the demand for high-value medicinal plants increased by 50%, while the availability declined by 26%⁴⁷



Need for Improvement in Primary Healthcare Infrastructure

- Number of centres has increased but not as much as projected population growth.⁷
- There is a shortfall of 74.2% at PHC's for Health Assistants in rural healthcare infrastructure.⁸
- As per the RHS 2021-22 report, India's 6,064 Community Health Centres (CHCs) are experiencing a significant shortage of approximately 80% in the number of required surgeons and paediatricians.⁸



Limitation in Tribal Healthcare Delivery⁴⁸

- There are several obstacles that hinder the successful delivery of healthcare services to these underserved populations. These challenges include geographical remoteness and limited infrastructure, language, and cultural barriers; and scarcity of healthcare professionals. Overcoming these challenges requires collaborative efforts between the government, medical specialists, and the indigenous tribes themselves.

⁴⁵ Gramvaani. (n.d.). Rural Health Care, Towards a Healthy Rural India.

⁴⁶ IBEF: High Demand For Medicinal Plants In India

⁴⁷ Rajya Sabha. (2022). Report No. 139 of the Standing Committee on Health and Family Welfare.

⁴⁸ National Health Systems Resource Centre. (2022). Report of the Expert Committee on Tribal Health.

Cross-Cutting Themes



Climate

Significance: Climate change has a significant impact on public health, leading to increased disease burden, malnutrition

Current Efforts: Government has launched various initiatives, such as National Action Plan on Climate Change and National Health Mission, to address the health impacts of climate change.

Way Forward: Health sector can integrate climate change adaptation and mitigation strategies into its policies and practices. This includes promoting renewable energy, implementing climate-resilient infrastructure, and raising awareness about the health impacts of climate change.



Education

Significance: It plays a crucial role in improving health outcomes by empowering individuals to make informed decisions about their health and well-being. It also helps in building a skilled workforce in the health sector.

Current Efforts: Government has implemented various programmes, such as the National Health Mission and the Ayushman Bharat scheme, to improve access to quality healthcare and promote health education.

Way Forward: Health sector can focus on strengthening health education at all levels, from schools to tertiary institutions. This includes integrating health education into the curriculum, providing training and professional development opportunities for healthcare professionals, and leveraging digital technology for distance learning.



Research and Development

Significance: It is essential for innovation and improving health outcomes. It helps in developing new treatments, vaccines, and diagnostic tools, as well as understanding the social and economic determinants of health.

Current Efforts: Government has established various research institutions and funding programmes, such as the Indian Council of Medical Research and the Department of Biotechnology, to promote R&D in the health sector.

Way Forward: Health sector can strengthen its R&D capabilities by increasing funding, promoting collaboration between academia, industry, and government, and focusing on priority areas such as infectious diseases, non-communicable diseases, and health systems research.



Financial Inclusion

Significance: Financial inclusion is crucial for ensuring access to affordable and quality healthcare services. It helps in reducing out-of-pocket expenses, improving health outcomes, and promoting social equity.

Current Efforts: Government has implemented various health insurance schemes, such as the Ayushman Bharat scheme, to provide financial protection to vulnerable populations.

Way Forward: India can continue to develop and implement policies that promote financial inclusion and leverage digital infrastructure to provide affordable and accessible financial services to underserved populations. The revised strategy must reach its target of 80 percent financial inclusion.



Digital Technology

Significance: Digital technology has the potential to transform the health sector by improving access to healthcare services, enhancing patient care, and enabling better health management.

Current Efforts: Government has launched various digital health initiatives, such as the National Digital Health Mission and the eSanjeevani telemedicine platform, to promote the use of digital technology in the health sector.

Way Forward: Health sector can focus on scaling up digital health solutions, ensuring data privacy and security, and promoting digital literacy among healthcare professionals and the general population.

Ministries and Departments

Ministry	Department and Bodies	Functions and Roles
 Ministry of Health and Family Welfare	Department of Health and Family Welfare	Responsible for overseeing and promoting public health initiatives. It plays a central role in formulating and implementing health policies, programmes, and regulations across the country. With a focus on both individual well-being and family welfare, the department strives to ensure accessible and quality healthcare services for all citizens
 Ministry of Housing And Urban Affairs	Central Public Works Department	It is a premier engineering organisation under the Government of India. Tasked with providing comprehensive construction and maintenance services, CPWD plays a crucial role in shaping the country's infrastructure landscape
 Ministry of Finance	Insurance Regulatory and Development Authority of India	It serves as the regulatory body overseeing the insurance sector in India. Its primary objective is to safeguard the interests of policyholders while promoting the growth and development of a robust and inclusive insurance industry
 Ministry of Tourism	Niche Tourism	It focuses on promoting and developing specialised tourism segments. This department caters to unique interests such as Eco and Wildlife, MICE, Sustainable, Cruise, Golf, Polo, Medical, Wellness
 Ministry of Environment, Forest and Climate Change	Central Pollution Control Board	Statutory body responsible for monitoring and controlling environmental pollution. It formulates policies and guidelines to address pollution issues and collaborates with state pollution control boards to enforce environmental regulations
 Ministry of Consumer Affairs, Food and Public Distribution	Department Of Food and Public Distribution	Responsible for formulating and implementing policies related to food grains, essential commodities, and public distribution systems. Its main focus is to ensure food security and availability of essential items

 Ministry of Rural Development	National Social Assistance Programme	Its welfare initiative that provides financial assistance to vulnerable and marginalized individuals and households. It aims to alleviate poverty and improve the quality of life by offering pensions and support to the elderly, widows, and disabled individuals
 Ministry of Commerce and Industry	National Accreditation Board for Hospitals and Healthcare Providers	It is responsible for setting and maintaining rigorous standards of quality and patient safety in the healthcare industry. It evaluates and accredits healthcare facilities based on various parameters to ensure excellence in healthcare delivery

Government Interventions*

Programme Name	Intervention
  Ayushman Bharat Programme Ayushman Bharat (AB) is an attempt to move from a selective approach to health care to deliver comprehensive range of services spanning preventive, promotive, curative, rehabilitative and palliative care.⁴⁹	It has two components which are complementary to each other. Under its first component, is Ayushman Arogya Mandir and other being Pradhan Mantri Janrogya Yojana. Other two components are Ayushman Bharat Digital Mission and Ayushman Bharat Health Infrastructure Mission.^{50,51}
      RMNCH+A Strategy RMNCH+A strategy is constructed around the idea of a continuum of care and is comprehensive in nature, covering all actions targeting reproductive, maternal, newborn, child, and adolescent health within a wide framework.⁵²	Interventions under this programme are Janani Suraksha Yojana, Janani-Shishu Suraksha Karyakram, Pradhan Mantri Surakshit Matritva Abhiyan, Surakshit Matritva Aashwasan (SUMAN), LaQshya, Dakshata
 Universal Immunization Programme (UIP) It is aimed at providing free vaccinations to all eligible children, pregnant women, and other high-risk groups. This programme plays a crucial role in preventing and controlling vaccine-preventable diseases, contributing to the reduction of infant mortality and morbidity rates.⁵³	Mission Indradhanush was introduced to achieve a full immunisation rate of over 90% for newborns by 2020, innovative and strategic methods were employed to ensure the vaccination of all children.
   Integrated Disease Surveillance Project (IDSP) It aims to strengthen disease surveillance and response systems. It operates at both national and state levels, facilitating early detection and containment of disease outbreaks.⁵⁴	Key interventions are Integrated Health Information Platform, Strategic Health Operations Centre and Health Emergency Operations Center
   Saksham Anganwadi and Poshan 2.0 It is an Integrated Nutrition Support Programme. It aims to address issues of malnutrition in children, adolescent girls, pregnant women and lactating mothers through a strategic shift in nutritional content⁵⁵	Jan Andolans mobilises various communities and sectors to foster a desire to eat nutrient-rich food. Poshan Tracker has facilitated migration services for expectant and nursing mothers between AWCs inside and outside of a State. IFA supplementation, including the supply of IFA tablets, referral services, and Nutrition and Health Education.

49 National Health Authority. (n.d). About PMJAY. Retrieved 1 September 2023

50 National Health Authority. (n.d). Ayushman Bharat Digital Health Mission. Retrieved 2 September 2023.

51 Ministry of Health and Family Welfare. (2022). Note on PM- Ayushman Bharat Health Infrastructure Mission.

52 National Health Mission. (n.d.). Maternal Health. Retrieved 1 October 2023.

53 National Health Mission. (n.d). Immunization. Retrieved 1 October 2023

54 Integrated Disease Surveillance Project. (n.d.). Home. Retrieved 1 October 2023

55 Women and Child Development. (2022). Saksham Anganwadi and Poshan 2.0 Guidelines.

Programme Name	Intervention
	Drug and Vaccine Distribution Management System DVDMS is a system that manages the purchase order, distribution, quality control, and monitoring of drugs and vaccines for various disease control programmes. It aims to provide quality drugs to the hospitals at a reasonable price and to ensure timely and transparent delivery to the beneficiaries by enabling data based informed decisions.⁵⁶
	Atal Jai Anusandhan Biotech (UNATI) Mission Interdisciplinary Group for Advanced Research on Birth Outcomes (GARBH-Ini)—DBT India Initiative Maternal and Child Health, and develop prediction tools for pre-term berth. IndCEPI is a mission to develop affordable vaccines for endemic diseases. Clean energy Mission is a Development of innovative technology interventions for Swachh Bharat. It consists of various sub-missions like GARBH-Ini for maternal and child health, Mission on Anti-Microbial Resistance, Ind Coalition for Epidemic Preparedness Innovations (CEPI) to develop infectious disease vaccines, Biofortification of food, and Clean.²⁸
	National AYUSH Mission (NAM) Ministry of AYUSH has incorporated eight programmes National Programme for Prevention and Management of Musculoskeletal Disorders. Integration Of AYUSH with National Programme for Prevention and Control of Cancer, Diabetes, Cardiovascular Diseases and Stroke. KARUNYA is to improve quality of life of patients under palliative care. SUPRAJA- Strengthening of AYUSH based Maternal and Neo-natal Interventions through AYUSH teaching hospitals. National Programme on AYUSH Morbidity Management and Disability Prevention of Lymphatic Filariasis AYURVIDYA is Programme for promotion of healthy lifestyles in 75,000 schools through AYUSH. Setting up of different capacity hospitals. It launched in 2014 is the flagship centrally sponsored mission through which AYUSH services are provided as part of Public Health Services. The primary objective of NAM is to promote AYUSH medical systems by providing cost-effective AYUSH services, strengthening educational systems, ensuring quality control of Ayurveda, Siddha, Unani, and Homoeopathy (ASU & H) drugs, and ensuring the sustainable availability of ASU & H raw materials.³⁸
	National Action Plan on AMR (NAP-AMR) The NAP-AMR was launched in 2017 to tackle AMR in India. It outlined a multi-sectoral approach to address the issue and focuses on key areas such as surveillance, rational use of antibiotics, infection prevention.⁵⁷ The primary objective of NAP-AMR is to actively address the challenge of AMR within India and actively participate in global endeavours to combat this significant public health concern. It aims to create and reinforce governance structures and enhance the capabilities of all involved parties to diminish the repercussions of AMR in India.

*There are many government interventions, selected ones have been described above.

⁵⁶ National Cold Chain and Vaccine Management Resource Centre. (n.d.). *NCCMIS Web Application*. Retrieved 18 September 2023.

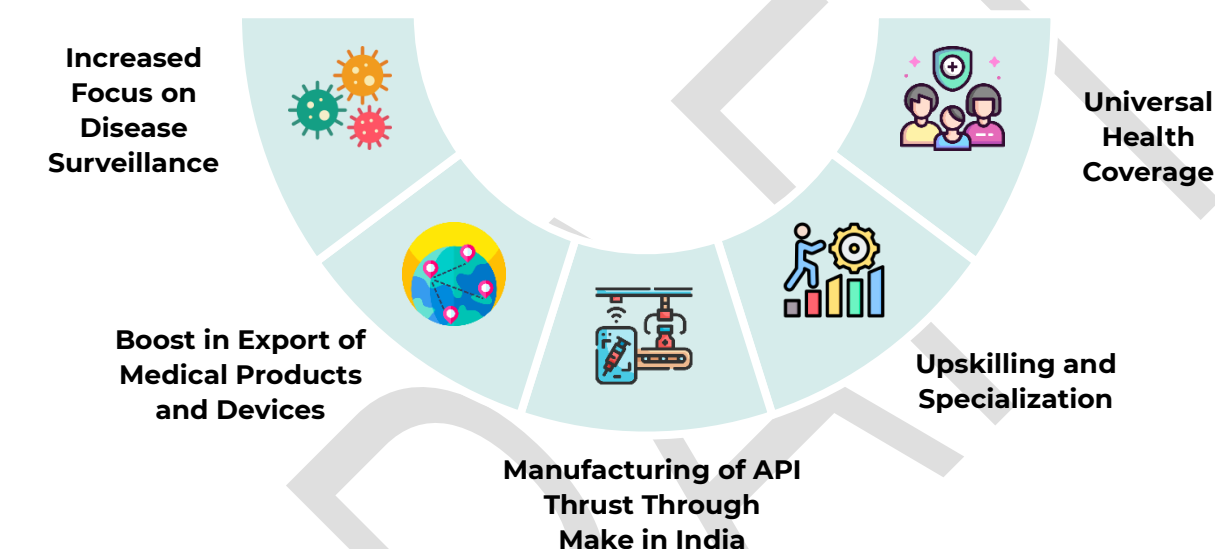
⁵⁷ National Centre for Disease Control. (2014). *National Action Plan, Antimicrobial Resistance*

Way Forward

In the next ten years, India has a special chance to change its healthcare system. Below summary gives a big picture of the findings and suggestions for ways to transform the system in India.

The health system's effectiveness in offering quality, affordable, and timely healthcare, as well as consistent care and benefit portability, is hindered by the significant disarray in risk pooling and healthcare provision. There's a need to find creative ways to involve those outside the formal system in contributory insurance. The general healthcare delivery approach should move towards more customer-focused and outcome-oriented integrated care models. In the near future, a priority should be placed on addressing urgent needs, particularly in emergency care, in rural and semi-urban parts of India.

Managing the diverse landscape of healthcare providers poses a challenge that can be enhanced by coordinated efforts between the public and private sectors.²



Universal Health Coverage (UHC)

- There has been an increased focus of the government to strengthen and upgrade existing infrastructure with specific focus on improving primary healthcare.⁵⁸
- Increased private hospitals empanelment in PM-JAY and increasing digital penetration is enhancing access to quality health care.⁵⁹

⁵⁸ Ministry of Information and Broadcasting. (2023). *Strengthening Access to Medical Education in India*.

⁵⁹ Research and Information System. (2021). *Ayush Sector in India: Prospects and Challenges*.

Upskilling and Specialization

- Government is looking to further increase PG seats and strengthen more specialization (like mental health).
- Non-Profit Organisation Echo India, collaborated with NHM Mizoram for conducting an 'Immersion Training' for senior health officials of the state.⁶⁰

Manufacturing Thrust Through Make in India

- Local production of medical devices and pharmaceutical APIs (active pharmaceutical ingredients) is reducing dependency on foreign countries.¹⁹
- Production-linked schemes and various schemes promoting tax rebates and incentives are encouraging industry to enhance local production.¹⁹

Boost in Export of Medical Products and Devices

- Export of alternative medicine has risen rapidly over the years and government has launched AYUSH Export Promotion Council (EPC) with an aim to promote exports of products of Ayush System⁶¹
- Exports of other products like medical devices are being promoted.²³
- COE and MedTech Expo 2023 launched to foster innovation and provide distinctive platform for promotion of medical devices.¹⁹

Increased Focus on Disease Surveillance

- Post-COVID, government is looking to improve preparedness of surveillance and warning system against emerging diseases.
- Programmes like disease surveillance and Antibiotic Stewardship Programme (AMSP) will gain more scale by incorporating more hospitals and more states.⁶²

⁶⁰ Directorate of Information & Public Relations. (2022). *Echo india in collaboration with National Health Mission (NHM) Mizoram*.

⁶¹ Ayush EXCIL. (n.d.). *AYUSH EXCIL*. Retrieved 10 October 2023.

⁶² ICMR. (n.d.). *Antimicrobial Stewardship Program (AMSP) curriculum*.

1. Mukhya Mantri Kshay Nivaran Yojana: State's Initiative to End TB (Himachal Pradesh)⁶³



Background

Tuberculosis stands as the foremost infectious disease responsible for fatalities on a global scale, and the same holds true for India. On the 24th of March 2018, the esteemed Chief Minister of Himachal Pradesh introduced the "Mukhya Mantri Kshay Rog Nivaran Yojna (MMKRNY)" with the goal of achieving the objective of eradicating Tuberculosis ahead of the predetermined timelines established by the Government of India. In order to expedite the efforts to eliminate TB, an additional budgetary allocation was deemed necessary, beyond the existing RNTCP (Revised National Tuberculosis Control Programme).



Steps taken by the Government

MMKRNY places its emphasis on preventive, investigative, and remedial actions.

- Committees for the elimination of TB were officially established at both the state and district levels. The Panchayati Raj Department and the Ayurveda Department were formally involved in these efforts.
- Sensitization initiatives were carried out for Panchayati Raj Institution (PRI) members, and a forum aimed at achieving TB-free Gram Panchayats was established. An extensive campaign for actively detecting TB cases was also executed.
- Training programmes for Medical Officers (MOs) and pharmacists within the Ayurveda Department included orientation sessions.
- The diagnostic services were broadened, with the procurement of five CBNAAT machines. Continuing Medical Education (CME) sessions were arranged for private practitioners and chemists. Additionally, the TB Mukht Himachal mobile application was created.



Impact generated

- A total of 5,527 Panchayati Raj Institutions (PRIs) representing 3,301 Gram Panchayats throughout the state were sensitized as part of MMKRNY. This programme effectively raised widespread awareness among various stakeholders with the aim of achieving a tuberculosis-free state.
- In the initial phase of the TB Mukht Himachal Abhiyan, nearly 500 new TB cases were identified and promptly initiated on treatment. This led to the state achieving the highest Annualized TB case notification rate in India for the year 2018.
- A combined total of 396 private practitioners and 930 chemists were sensitized. This resulted in a notable 61% surge in TB case reporting from the private sector in 2018.
- A comprehensive count of 692 Ayurveda Medical Officers and 1027 Ayurveda Pharmacists received training as part of the programme. The Ayurveda department's role in both identifying cases and managing patients has been on the rise. The expansion of CBNAAT facilities led to a substantial 72% achievement in Universal Drug Susceptibility Testing (DST).

⁶³ National Health Systems Resource. (2019). *We Care - Good, Replicable & Innovative Practices*.



2. Aardram Mission Implementation (Kerala)⁶⁴



Background

Deliver readily available, universally accessible, and cost-free high-quality healthcare services at primary health facilities to ensure the well-being and health of individuals, while also establishing standardized referral procedures.



Steps taken by the Government

- 'Aardram Mission', is a state government effort to make government hospitals people-friendly by improving basic infrastructure and quality of services by upgrading Primary Health Centres (PHCs) as Family Health Centres (FHC).
- FHCs are equipped with a laboratory, pharmacy, well-attended OP counters, fitness centre and yoga training facility. Field staff at FHCs are trained on NCDs and CD-related health issues and management. Team building training consisting of people's representatives and all health staff (including doctors and field staff) are also being conducted for facilitation in providing quality services in govt. health institutions.
- As a part of wellness, the State has planned for "Sampoorna Yoga Keralam," which envisages the propagation of yoga. It is proposed to disseminate yoga training to the public by empowering ASHA, school volunteers, and community volunteers through continuous yoga training.
- Muhamma Grama Panchayat is declared as Sampoorna Yoga Gramam (23,575 people trained out of 30,000 population) in Muhamma, Alappuzha District. State has upgraded 170 PHCs to FHCs in 2017-18 and another 500 PHCs in 2018-19. In current year, target is to cover all the remaining PHCs.
- Under Sampoorna Yoga Keralam, aim is to empower Yoga Volunteers to train public, school children, etc. through six sessions/ month, extending within one-two years to cover the entire ward, school and public (covering all 941 panchayats).

⁶⁴ [Arogya Keralam. \(n.d.\). Aardram. Retrieved 10 October 2023](#)



Impact generated

- Ownership of health institutions among Panchayati Raj Institutions and general public is clear, with institutions augmenting more funds for health institutions over and above the budget allotted through budgetary support.
- Funds under Members of Parliament Local Area Development Scheme (MPLADS), Members of Legislative Assembly Local Area Development Scheme (MLALADS), Panchayat funds, CSR funds etc. are being tapped for the upgradation of health institutions.



3. Marching Ahead in achieving Universal Health Coverage: A Milestone under Sustainable Development Goals (SDGs) (Odisha)⁶⁴



Background

Odisha faced several challenges such as high out-of-pocket expenditures despite high utilisation of the public sector, inadequate public healthcare infrastructure, HR shortage, limited use of advanced medical technology, lack of assured medicines and diagnostic services, and lack of continuum of care across levels of care



Steps taken by the Government

During the last one decade, Odisha has introduced several reforms for strengthening the delivery of health services through outcome-based, participatory planning process and additional resource allocation from State budget.

Based on the guiding principles- “Sustha Odisha, Sukhi Odisha” and “Healthcare for All – Each Life matters”, over 40 free and assured health care schemes were introduced under National Health Mission and with a steady increase of health financing under the State Budget. The share of health budget as % of GSDP has been increased from 0.89% in 2005-2006 to 1.48% in 2018-2019.

Some of the key initiatives at state level include -mobile health units under KBK; Sishu Abong Matru Mrutyuhara Purna Nirakarana Abhijana (SAMMPurNA), Free Cancer Care Programme - Day Care Chemotherapy Center at DH, Corpus Fund for Human Resource in KBK+ areas; Incentive for HR in difficult areas, reimbursement of transportation cost at INR 1,000/- to all pregnant women for institutional delivery in notified villages; Durgama Anchalare Malaria Nirakaran- DAMaN; ASHA Kalyan Yojana; establishment of new Medical and Nursing Colleges; Universal Eye Healthcare Programme; Cash incentives at INR 500/- per month to all Blood Disorder for continuing treatment; Free Specialists Services at Urban PHCs/CHCs under Ama Clinic; Strengthening Ancillary Services in Public Facilities; Drop back assistance of INR 500/- to pregnant women and sick infants; Digital Dispensaries through telemedicine in difficult areas; Free Blood Services to all at Government Blood Banks; and Free Healthcare Services for all at all public health facilities up to Medical under the Biju Swasthya Kalyan Yojana (BSKY) with

cashless assistance of INR 5 lakhs per family per year and additional INR 10 lakhs for the women members of the beneficiaries' families.



Impact generated

There is a marked trend of improved utilization of health care services. For instance:

- Over 3.3 crores patients received free medicines since 2015.
- Annually over 1.50 crores patients received Free Diagnostic Services
- Over 60,000 dialysis sessions conducted since 1st January 2018
- Over 2,95,587 free surgeries conducted since 15th August 2018.
- Over 62,000 patients received free cancer consultation services and 22,715 patients received chemotherapy and 5,659 patients received radiotherapy sessions conducted since August, 2018
- Primary health care services provided in 6,251 difficult areas through 177 Mobile Health Units
- Annually over 10 lakhs pregnant women and sick newborns benefitted under SAMMpurNA
- Increased MBBS seats from 450 to 1,150 through 4 new Medical Colleges.;
- Creation of 2,508 posts of doctors, 400 posts of Dental surgeons, and 5719 posts of paramedics.
- Around 70 lakhs families (3.5 crore people) are covered under BSKY.



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